

Other dietary recommendations

The gluten-free diet (GFD) is the foundation of therapy for dermatitis herpetiformis (DH). However, patients should avoid excessive iodine intake (e.g., seafood, multivitamin-multimicronutrient supplements) as it may trigger disease relapses, particularly in those on an incomplete GFD.

Dapsone

Dapsone is the first-line drug for symptomatic treatment of DH. Although no randomized controlled trials have evaluated its efficacy—except for small case series—there is strong consensus that dapsone is highly effective. It remains the most effective treatment for cutaneous symptoms and pruritus until the GFD takes full effect, which may take 6–24 months. Skin lesions and itching typically resolve within 3–4 days of starting dapsone, and symptoms usually recur within days of discontinuation.

Before initiating dapsone, serum glucose-6-phosphate dehydrogenase (G6PDH) activity should be assessed to rule out deficiency and prevent hemolytic anemia. The starting dose depends on symptom severity. A low-dose regimen begins at 25 mg once or twice daily to minimize side effects, with weekly increments of 25 mg until an effective dose is reached, up to a maximum of 200 mg/day. For maintenance, a dose of 0.5–1 mg/kg/day is generally sufficient to control pruritus and prevent new lesions.

Dietary counselling

All DH patients should be referred to a dietitian to ensure proper implementation of a gluten-free diet, especially if additional dietary modifications are required due to comorbidities.

Indications of dapsone treatment in DH patients

Dapsone therapy is indicated in the following situations:

- Intolerable or severe skin involvement
- Skin manifestations persist despite a strict GFD
- Patient is unwilling or unable to adhere to a GFD

In cases of severe disease and intense pruritus, higher starting doses (e.g., 50 mg twice or three times daily) may be used, provided G6PDH deficiency has been excluded. Once clinical remission is achieved, the dose should be tapered to the lowest effective maintenance level. If no improvement is observed within one week at a dose ≥ 150 mg/day, the diagnosis should be re-evaluated or alternative therapies considered. A strict and sustained GFD allows for rapid reduction of dapsone dosage, and in most patients, dapsone can eventually be discontinued entirely.

For pediatric patients, the initial dose is 1–2 mg/kg orally once daily or in two divided doses, with a maximum single dose of 50 mg. The maintenance dose is individualized and typically ranges from 10–25% of the initial effective dose.

No dose adjustment is necessary in renal impairment. In hepatic impairment, caution is advised, and regular laboratory monitoring is recommended.

Dapsone therapy should continue until complete remission is achieved. In case of relapse, the dose may be temporarily increased by the patient under medical

guidance.

restarted.

If a severe relapse occurs after discontinuation, dapsonone can be safely